



REFERRAL PACKET

When chronic symptoms don't respond to standard care, the jaw is often the missing variable.

A clinical orientation for referring physicians, dentists, and allied practitioners.

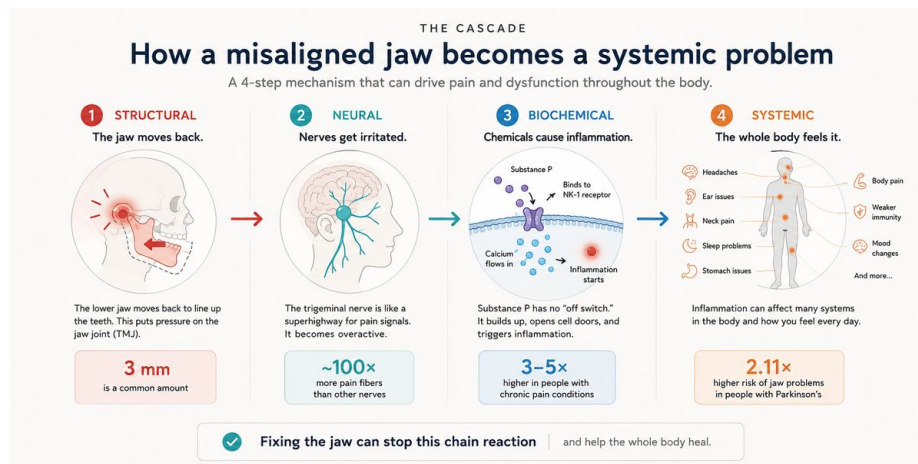
Dr. Dwight Jennings runs a tertiary referral practice in the San Francisco Bay Area, focused on jaw orthopedics and the cranio-facial / systemic-symptom interface. Patients arrive with chronic headaches, fibromyalgia, sleep apnea, TMJ dysfunction, and undiagnosed pain syndromes that have not improved under mainstream care. A meaningful percentage of his patients fly in from outside California.

The framework is straightforward: a misaligned bite irritates the trigeminal nerve; chronic trigeminal activation produces a constellation of downstream symptoms that mainstream medicine commonly treats as separate diagnoses. Realigning the bite — almost never surgical — quiets that irritation, and in responders the constellation reduces or resolves.

“Curiously, I have never heard a single physician mention the idea of looking at the bite on patients having broad medical complaints.” — Dr. Jennings

How a misaligned jaw becomes a systemic problem

Four steps. The first three are established neuroscience. The fourth maps to forty years of Dr. Jennings' clinical observation.



- 1 Structural · Jaw misalignment.** A vertical-height deficiency — often inherited, often worsened by extraction-based orthodontics — forces the lower jaw to retrude to bring the teeth together. The TMJ is compressed; the chewing muscles work continuously to hold the jaw out of rest.
- 2 Neural · Trigeminal hyperactivation.** Chronic muscle and joint irritation activates the trigeminal nerve — the densest concentration of nociceptive C-fibers in the body. The nerve releases **substance P**, a neuropeptide with no reuptake mechanism. It accumulates and lingers.



- 3 Biochemical · Substance P at the cell.** Substance P binds the NK-1 receptor, opens cell membranes, lowers cell voltage, and allows calcium influx. It is established as a primary driver of inflammatory processes in the respiratory, gastrointestinal, and musculoskeletal systems.
- 4 Systemic · Downstream dysfunction.** Sustained substance P elevation correlates clinically with the conditions Dr. Jennings sees most often: chronic headaches, fibromyalgia, IBS, autoimmune disorders, sleep apnea, movement disorders, mood and ADD/ADHD presentations, and a broader pattern of complex chronic illness.

The treatment is undramatic. Almost never surgery. A precision-fitted removable dental appliance repositions the mandible to a physiologically optimal rest position. Substance P production downregulates. In responders, the downstream symptoms reduce or resolve.

Who to refer

Dr. Jennings is not a general dentist — he is a tertiary specialist for patients who have plateaued under conventional care. Conditions where his clinical experience suggests the bite is worth examining:

TMJ pain & dysfunction. Jaw pain, clicking, locking, ear pressure, facial pain; post-surgical TMJ complications. The original specialty of the practice.

Chronic headaches & migraines. Daily or near-daily headaches; classical migraine; tension-type or cluster patterns unresponsive to standard pharmacological care. Bite alignment is reported at ~85% effective across types in the published literature.

Sleep apnea (and CPAP-intolerant). Particularly when CPAP has failed or been declined; when snoring, retrognathia, and chronic fatigue co-occur; or when sleep apnea presents alongside the broader chronic-illness pattern.

Movement disorders. Parkinson's, Tourette's, dystonia, torticollis, gait disorders. Not all cases respond — but a subset have a strong cranio-mandibular component and improve with alignment therapy when other approaches have not.

Surgical alternative. Patients recommended for TMJ joint replacement, condylectomy, or jaw-advancement surgery deserve a non-surgical second opinion. Many can be treated non-surgically with precision jaw orthopedics.

Complex & undiagnosed. Fibromyalgia, IBS, autoimmune patterns, chronic fatigue, ADD/ADHD, anxiety and depression that don't fit cleanly into a single specialty's workup. Often these patients have seen many physicians and run a long differential.

How to refer

There is no formal referral form. A short note from you, plus the patient calling to schedule, is enough.



What to send

Helpful before the first visit: panoramic dental imaging, recent MRI or CT of head and neck, sleep study, current medication list, and a brief narrative of what has been tried.

Who is a good fit

Patients who have plateaued under conventional care and present with TMJ pain or dysfunction; chronic headache or migraine; a bite that has shifted with age or after orthodontics; chronic illness with an inflammatory or neuropeptide-mediated footprint; sleep apnea with retrognathia or CPAP intolerance; or any movement disorder where a trigeminal-mandibular contribution has not been ruled out.

Who is not a good fit

Cosmetic dentistry, routine restorative work, adolescent orthodontics without complicating symptoms, or acute dental emergencies. Dr. Jennings does not provide general dentistry.

Logistics

Phone	(510) 522-6828
Email	dejdds@gmail.com
Address	2187 Harbor Bay Pkwy. Alameda, CA 94502
Hours	Mon–Thu, 7 AM – 4 PM
Airport	Oakland International (OAK), 5 min from office
Website	tmjcalifornia.expertaisys.com

What happens after the referral

A typical timeline once the patient calls to schedule. Treatment is a process, not a procedure — clinical responses commonly appear within the first three months, with stabilization at six to nine months.

WEEK 0 Initial consult. Comprehensive history, exam, panoramic imaging if not provided. ~90 minutes.	WEEK 1–2 Diagnostics. TMJ imaging, kinesiographic jaw motion, dental models, intra-oral photography.	WEEK 2–4 Appliance fitting. Precision-fitted removable appliance fabricated and adjusted to physiological rest.	MONTH 1–6 Active treatment. Periodic refinements as the mandible accommodates. Symptom response tracked monthly.	MONTH 6–9 Stabilization. Final TENS-monitored bite refinement; transition to maintenance protocol.
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This packet is informational. Nothing here constitutes diagnosis or treatment advice. Any clinical decision remains with the patient and their care team.